

Potential Donor?

Mechanically
Ventilated?

Y

N



GCS < 4?

N



Consider early referral to
Transplant Coordinator to
discuss suitability:
<https://bit.ly/EarlyTcReferral>

Y

Neuro review: Is the patient
for neurosurgical
intervention?



Y

N

Counsel family on poor prognosis
and intent to conduct brainstem
death testing

Conduct brainstem death
testing.
Brain dead?

Y

N

Refer to Transplant
Coordinators:
<https://bit.ly/TcRef>

Re-evaluate, only ancillary
testing if clinical tests not
possible

Inform family of diagnosis.
Counsel on no chance of
recovery. **DO NOT DISCUSS
ORGAN DONATION**

Notify Transplant
Coordinators to **prep** for OD
conversation
<https://bit.ly/TcRef>

Does the family accept the
confirmation of death?

N

Provide the family with specific timeframe
to process (<30mins); do not withdraw
support immediately. Initiate counseling
again

Y

Notify Transplant
Coordinators to **intiate** OD
conversation
<https://bit.ly/TcRef>

1. Complete **FPS 100**
2. Contact FPS (**021 931 4232**) to discuss
(w/ Transplant Coordinator)
3. Contact SAPS Liaison (021 938 4984)

Y

Family Consents to OD?

N



Y

Unnatural death?

N

Complete
DHA1663-B1163

Keep door stable and asisst
with handover to LifePod

Unsure?



Get help



1. Identify potential donor. Meets the following criteria:
 - a. If mechanically ventilated **AND ANY OF THE FOLLOWING:**
 - i. There is a catastrophic (can confirm with Neuro in next step) brain injury with GCS 3-4 (which cannot be explained by sedation) after completion of a period of observation (+24hrs) to allow prognostication.
 - ii. A shared decision has been made to perform brain death tests.
2. Consider referral to T/c for help determining suitability
 - a. On WhatsApp / Call: 063 420 4395
 - b. Click this link for to send a template message to them: <https://bit.ly/EarlyTcReferral>
3. Refer to Neurosurgery for review of CT-brain to confirm extent of injury
 - a. If catastrophic and poor prognosis >> Continue flow
 - b. If not catastrophic but poor prognosis>> Counsel the family on prognosis, continue critical care support. Reevaluate
 - c. If for neurosurgical intervention >> Exit flow
4. Counsel Family
 - a. On poor prognosis
 - b. Intent to conduct brainstem death testing to confirm
5. Conduct brainstem death testing
 - a. x2 doctors; one who has been practising for at least five years, not an intern
 - b. See attached check-list
 - c. If Yes >> Continue flow
 - d. If No >> Continue critical care support and reevaluate, only conduct ancillary testing if clinical testing cannot be done
6. Refer to the transplant coordinators
 - a. Telephonically on:
 - b. On WhatsApp / Call: 063 420 4395
 - c. Request presence and help for case, click this link for a template message: <https://bit.ly/TcRef>
7. Inform the family of the diagnosis of brain death
 - a. Do not discuss / mention OD at all yet
 - b. Ensure that the family fully understands and accepts diagnosis and the lack of any prognosis / recovery
 - c. Notify transplant coordinators so that they can begin counseling
 - i. If family consents >> Continue flow
 - ii. If family refuses >> Exit flow
8. Finalise referral
 - a. If unnatural death: Complete FPS100
 - i. Call warrant officer / SAPS liaison if unsure: (021 938 4984).
 - ii. Assist Transplant coordinators in contacting FPS (contact: 021 931 4232) to determine which organs may be procured
 - b. If natural: Complete DHA1663-B1163
 - i. Ensure the form accompanies the donor
 - c. Ensure donor remains stable pending handover